PATIENT NAME : MR.RAJEEV KAKKAR

REF. DOCTOR :

FLT LT RAJAN DHAL CHARITABLE TRUST IPD
SECTOR-B, POCKET-1, ARUNA ASAF ALI
MARGVASANT KUNJ,
NEW DELHI 110070
11-42776222

ACCESSION NO : **0013YL007097**
PATIENT ID : FH.10693597
CLIENT PATIENT ID: UID:10693597
ABHA NO :

AGE/SEX : 63 Years Male
DRAWN : 23/12/2025 02:11:00
RECEIVED : 23/12/2025 06:44:31
REPORTED : 23/12/2025 08:42:46

CLINICAL INFORMATION :

UID:10693597 REQNO-24087368
IPD-WARD-B1
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Final**

HAEMATOLOGY - CBC

CBC WITH PS, EDTA BLOOD BLOOD

BLOOD COUNTS,EDTA WHOLE BLOOD

BLOOD COUNTS,EDTA WHOLE BLOOD	Results	0013YL006731 22/12/25 04:57	0013YL006566 21/12/25 05:11	0013YL006247 20/12/25 04:56	Biological Reference Interval	Units
HEMOGLOBIN (HB) METHOD : SLS- HEMOGLOBIN DETECTION METHOD	12.9 L	12.7	13.6	14.6	13.0 - 17.0	g/dL
RED BLOOD CELL (RBC) COUNT METHOD : IMPEDANCE / MICROSCOPY	6.14 H	6.10	6.29	6.68	4.5 - 5.5	mil/µL
WHITE BLOOD CELL (WBC) COUNT METHOD : FLOWCYTOMETRY	[illegible] H	20.95	19.70	21.93	4.0 - 10.0	thou/µL
PLATELET COUNT METHOD : IMPEDANCE / MICROSCOPY	241	257	208	200	150 - 410	thou/µL

RBC AND PLATELET INDICES	Results	0013YL006731 22/12/25 04:57	0013YL006566 21/12/25 05:11	0013YL006247 20/12/25 04:56	Biological Reference Interval	Units
HEMATOCRIT (PCV) METHOD : DC DETECTION	42.10	42.10	45.50	47.90	40.0 - 50.0	%
MEAN CORPUSCULAR VOLUME (MCV) METHOD : DERIVED FROM IMPEDANCE IN ASURE	68.6 L	69.0	72.3	71.7	83.0 - 101.0	fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	21.0 L	20.8	21.6	21.9	27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION (MCHC) METHOD : CALCULATED	30.6 L	30.2	29.9	30.5	31.5 - 34.5	g/dL

Dr Nitin Kumar Dumeer, M.D. FRCPATH Histopathology (Reg No. DMC 30700)
Lab Head

Dr.Ruchika Majoka
Consultant Pathologist

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ULR No.13000002452123-0013

NC-5836

PATIENT NAME : MR.RAJEEV KAKKAR | **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD
SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ,
NEW DELHI 110070
11-42776222

ACCESSION NO : **0013YL007097**
PATIENT ID : FH.10693597
CLIENT PATIENT ID: UID:10693597
ABHA NO :

AGE/SEX : 63 Years Male
DRAWN : 23/12/2025 02:13:00
RECEIVED : 23/12/2025 06:44:33
REPORTED : 23/12/2025 08:42:46

CLINICAL INFORMATION :

UID:10693597 REQNO-24087368
IPD-WARD-B1
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Final**	Results			Biological Reference Interval	Units
RED CELL DISTRIBUTION WIDTH (RDW) METHOD : DERIVED FROM IMPEDANCE MEASURE	15.7 H	15.8	16.3	17.0	11.6 - 14.0	%
MEAN PLATELET VOLUME (MPV) METHOD : DERIVED FROM IMPEDANCE MEASURE	10.3	11.6	11.2	11.0	6.8 - 10.9	fL

WBC DIFFERENTIAL COUNT	Results	0013YL006731 22/12/25 04:57	0013YL006500 21/12/25 05:11	0013YL006247 20/12/25 04:58	Biological Reference Interval	Units
NEUTROPHILS METHOD : FLOW CYTOMETRY & MICROSCOPY	86 H	87	86	87	40 - 80	%
LYMPHOCYTES METHOD : FLOW CYTOMETRY & MICROSCOPY	8 L	5	6	5	20 - 40	%
MONOCYTES	3	4	4	4	2 - 10	%
EOSINOPHILS METHOD : FLOW CYTOMETRY & MICROSCOPY	1	1	1	1	1 - 6	%
BASOPHILS METHOD : FLOW CYTOMETRY & MICROSCOPY	0	0	0	0	0 - 1	%
ABSOLUTE NEUTROPHIL COUNT METHOD : CALCULATED PARAMETER	19.15 H				2.0 - 7.0	thou/µL
ABSOLUTE LYMPHOCYTE COUNT METHOD : CALCULATED PARAMETER	1.56				1.0 - 3.0	thou/µL
ABSOLUTE MONOCYTE COUNT METHOD : CALCULATED	0.67				0.2 - 1.0	thou/µL
ABSOLUTE EOSINOPHIL COUNT METHOD : CALCULATED	0.22				0.02 - 0.50	thou/µL
ABSOLUTE BASOPHIL COUNT METHOD : CALCULATED	0				0.0 - 0.1	thou/µL

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Lab Head

Dr.Ruchika Majoka
Consultant Pathologist

View Details View Report

ULR No.13000002452123-0013

NC-5936

PATIENT NAME : MR.RAJEEV KAKKAR **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ, NEW DELHI 110070 11-42776222	ACCESSION NO : **0013YL007097** PATIENT ID : PH.10693597 CLIENT PATIENT ID: UID: 10693597 ABHA NO:	AGE/SEX : 61 Years Male DRAWN : 23/12/2025 02:13:00 RECEIVED : 23/12/2025 06:44:33 REPORTED : 23/12/2025 08:42:46

CLINICAL INFORMATION :

UID:10693597 REQNO-24087368
IPD-WARD-B1
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Final**	Results				Biological Reference Interval	Units
BAND (STAB) CELLS	2	2	2	7	0 - 5	%

***This is a tabular depiction of test results illustrating values of the previous three reports of the patient which needs to be correlated with the original reports**

Interpretation(s)

BLOOD COUNTS,EDTA WHOLE BLOOD-The RBC morphology is well preserved for 24hrs. However after 24-48 hrs a progressive increase in MCV and MCT is observed leading to a decrease in MCHC. A direct smear is recommended for an accurate differential count and for examination of RBC morphology.

RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate causes of iron deficiency anemia (<13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and confirmatory evaluation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.

WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age > 49.5 years old and NLR > 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease.

(Reference to : The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients ; A.-P. Yang, et al.; International Immunopharmacology 84 (2020) 106504)

This ratio element is a calculated parameter and out of NABL scope.

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Lab Head

Dr.Ruchika Majoka
Consultant Pathologist

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ULR No.1300000245212 3-0013

MC-5836

PATIENT NAME : MR.RAJEEV KAKKAR | **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ, NEW DELHI 110070 11-42776222	ACCESSION NO : **0013YL007097** PATIENT ID : FH.10693597 CLIENT PATIENT ID: UID 10693597 ABHA NO	AGE/SEX : 63 Years Male DRAWN : 23/12/2025 02:13:00 RECEIVED : 23/12/2025 06:44:31 REPORTED : 23/12/2025 08:42:46

CLINICAL INFORMATION :

UID:10693597 REQNO-24087368
IPD-WARD-B1
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Final**	Results	Biological Reference Interval	Units
BIOCHEMISTRY			
BLOOD UREA NITROGEN (BUN), SERUM			
BLOOD UREA NITROGEN METHOD : UREASE - UV	12	8 - 23	mg/dL
POTASSIUM, SERUM			
POTASSIUM, SERUM METHOD : ISE DIRECT	4.14	3.5 - 5.3	mmol/L
SODIUM, SERUM			
SODIUM, SERUM METHOD : ISE DIRECT	142	135 - 145	mmol/L
CREATININE EGFR- EPI,SERUM			
CREATININE METHOD : ALKALINE PICRATE KINETIC RATE BLANKED	0.84	0.80 - 1.30	mg/dL
AGE	63		years
GLOMERULAR FILTRATION RATE (MALE)	97.99		mL/min/1.73m2

Interpretation(s)

eGFR (ml/min/1.73 sq.meters)	Interpretation
>or= 90	Normal
60 - 89	Mild Decrease in GFR
30 - 59	Moderate Decrease in GFR
15 - 29	Severe Decrease in GFR
<15	End stage renal failure

- Kidney disease outcomes quality initiative (KDOQI) guidelines state that estimation of GFR is the best overall indices of the Kidney function
- It gives a rough measure of number of functioning nephrons .Reduction in GFR implies progression of underlying disease.
- The GFR is a calculation based on serum creatinine test.
- Creatinine is mainly derived from the metabolism of creatine in muscle, and its generation is proportional to the total muscle mass. As a result, mean creatinine generation is higher in men than in women, in younger than in older individuals, and in blacks than in whites.
- Creatinine is filtered from the blood by the kidneys and excreted into urine at a relatively steady rate.
- When kidney function is compromised, excretion of creatinine decreases with a consequent increase in blood creatinine levels. With the creatinine test, a reasonable estimate of the actual GFR can be determined.
- This equation takes into account several factors that impact creatinine production, including age, gender, and race.
- CKD EPI (Chronic kidney disease epidemiology collaboration) equation performed better than MDRD equation especially when GFR is high

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ULR No. 1300000245212 3-0013

MC-5836

PATIENT NAME : MR.RAJEEV KAKKAR | **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ, NEW DELHI 110070 11-42776222	ACCESSION NO : **0013YL007097** PATIENT ID : FH.10693597 CLIENT PATIENT ID: UID:10693597 ABHA NO :	AGE/SEX : 61 Years Male DRAWN : 23/12/2025 02:13:00 RECEIVED : 23/12/2025 06:44:31 REPORTED : 23/12/2025 08:42:46

CLINICAL INFORMATION :

UID:10693597 REQNO-24087368
IPD-WARD-B1
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Final**	Results	Biological Reference Interval	Units

>60 ml/min per 1.73m2). This formula has less bias and greater accuracy which helps in early diagnosis and also reduces the rate of false positive diagnosis of CKD.

References.

National Kidney Foundation (NKF) and the American Society of Nephrology (ASN).
Estimated GFR Calculated Using the CKD-EPI equation [illegible]
Khoman JK, et al. Impact of Removing Race Variable on CKD Classification Using the Creatinine-Based 2021 CKD-EPI Equation. Kidney Med 2022. 4:100471. 35756325
Harrison Principle of Internal Medicine, 21st ed. pg 62 and 334

Interpretation(s)
BLOOD UREA NITROGEN (BUN), SERUM-**Causes of Increased** levels include Pre renal (High protein diet, Increased protein catabolism, GI hemorrhage, Cardiac, Dehydration, CHF Renal), Renal Failure, Post Renal (Malignancy, Nephrolithiasis, Prostatism)
Causes of decreased level include Liver disease, SIADH.
POTASSIUM, SERUM-Hypokalemia (low K) is common in vomiting, diarrhea, alcoholism, folic acid deficiency and primary aldosteronism. Hyperkalemia may be seen in end-stage renal failure, hemolysis, trauma, Addisons disease, metabolic acidosis, acute starvation, dehydration, and with rapid K infusion.
SODIUM, SERUM-Increased in dehydration, cushings syndrome, aldosteronism; Decreased in Addisons disease, hypopituitarism, liver disease.

****End Of Report****
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Dr Nitin Kumar Dumeer, M.D. FRCPATH Histopathology (Reg No. DMC 30700)
Lab Head

Dr.Ruchika Majoka
Consultant Pathologist

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ULR No.1300000245212 3-0013

Fortis
FLT. LT. RAJAN DHALL
HOSPITAL

Mr. Rajeev Kakkar
63 Y/M; UHID: 10693597; IPID: 179296/25/130
BED: HDU 1; Domestic Insura; Dr.Neeraj Chaudhary/ Dr Nitin Kaushik/ Dr Hariom Sheor

'HIGH VALUE DRUGS' CONSENT FORM FOR PATIENT CONSUMPTION

Patient Name : MR. Rajeev kakkar

IPID _UHID : 179296 / 10693597

Department : ICU-6

Item detail:

Name of drug	Cost of drug per unit (Rs. Approx.)	Qty required
INJ UTRIP 2LAKH IU.	20640	4

Remarks:

Patient/ Responsible party's name

Signature

Sister Incharge's name & Signature

Physician's name & Signature

Front Office Supervisor (On duty)

NC-5836

PATIENT NAME : MR.RAJEEV KAKKAR | **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD
SECTOR-B, POCKET-1, ARUNA ASAF ALI
MARGVASANT KUNJ,
NEW DELHI 110070
11-42776222

ACCESSION NO : **0013YL005860**
PATIENT ID : FH.10693597
CLIENT PATIENT ID: UID:10693597
ABHA NO :

AGE/SEX : 63 Years Male
DRAWN : 18/12/2025 20:24:00
RECEIVED : 18/12/2025 20:53:33
REPORTED : 20/12/2025 12:09:03

CLINICAL INFORMATION :

UID:10693597 REQNO-24041022
IPD-ICU(2)
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Preliminary**	Results	Biological Reference Interval	Units

MICRO BIOLOGY

BLOOD CULTURE AEROBIC - AUTOMATED

BLOOD CULTURE - PRELIM

CULTURE	BLOOD (LEFT PERIPHERAL SITE)		
METHOD : FLUORESCENCE BASED CONTINUOUSLY MONITORING SYSTEM & AEROBIC CULTURE & BIOCHEMICALS OTHER TESTS			
ORGANISM	STERILE AFTER OVERNIGHT INCUBATION AT 37°C.		
METHOD : FLUORESCENCE BASED CONTINUOUSLY MONITORING SYSTEM & AEROBIC CULTURE & BIOCHEMICALS OTHER TESTS			
BLOOD CULTURE - FINAL	RESULT PENDING		

Interpretation(s)
BLOOD CULTURE - PRELIM-Blood is normally sterile. Bacteria enter the blood either through lymphatics from an infected site or due to intravascular infections. Transient bacteraemia occurs when normal bacterial flora is introduced into the blood e.g. following brushing of teeth. Intermittent bacteraemia results when bacteria from an extravascular infected site are spasmodically released into blood. Continous bacteraemia occurs where the organisms have direct access to the blood stream. Septicaemia ensues when the circulating bacteria multiply at a rate exceeding removal by phagocytosis. Pyaemia is a condition where pyogenic bacteria produce septicemia with multiple abscesses in internal organs. Septicemia and pyaemias are life threatening emergencies, thus making early recognition critical.

Method of Testing: Automated continuous monitoring blood culture system

Notes:

- The successful recovery of organisms from blood depends on many factors, such as the type of bacteremia, specimen collection method, blood volume, number and timing of blood cultures etc.
- Bacteria may not be continuously present in blood. Cultures should be performed as soon as possible after onset of chills or fever and before antimicrobial therapy is started. If possible, 2 to 3 specimens should be cultured at least 30- 60 minutes apart. The time interval can be shortened if there is an urgent need to begin therapy.
- Drawing a single blood culture set is not recommended as it can give both false negative (as seen in intermittent bacteremia) as well as false-positive results (due to contamination with skin flora).
- The greater the amount of blood added, greater the chance of recovering the organism. A minimum of 10 ml blood in adults and 1-5 ml in infants & children should be inoculated in each bottle.
- Blood cultures can be negative in patients on antibiotics. In this scenario, use of bottles containing resin is recommended. The resins neutralize the antibiotic to optimise microbial growth.
- Isolation of Salmonella typhi / paratyphi from blood is considered definitive diagnosis of enteric fever. Blood culture is usually positive in 90 % cases in the 1st week of infection, 75 % cases in the 2nd week, 60 % in the 3rd week, and reduces to 25 % thereafter. Multiple blood culture sets may help to increase the positivity rate. Blood cultures may be positive in only up to 40 % of patients on antibiotics.
- Note that blood culture detects bacterial causes of fever of unknown origin. A negative blood culture report does not rule out other infective causes such as viruses, mycobacteria, fungi, parasites, rickettsia, chlamydia, spirochaetes etc and non-infective causes.

End Of Report

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Senior Microbiologist

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ULR No.13000002450676-0013

MC-5836

PATIENT NAME : MR.RAJEEV KAKKAR **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ, NEW DELHI 110070 11-42776222	ACCESSION NO : **0013YL005861** PATIENT ID : FH.10693597 CLIENT PATIENT ID: UID:10693597 ABHA NO :	AGE/SEX :63 Years Male DRAWN :18/12/2025 20:24:00 RECEIVED : 18/12/2025 20:54:47 REPORTED :21/12/2025 11:25:50

CLINICAL INFORMATION :

UID:10693597 REQNO-24041022
IPD-ICU(2)
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Preliminary**	Results	Biological Reference Interval	Units

MICRO BIOLOGY

HIGH RISK INFECTION SURVEILLANCE PACKAGE

CULTURE; URINE

SPECIMEN SOURCE	URINE
METHOD : AEROBIC CULTURE & BIOCHEMICAL & OTHER TESTS	
CULTURE	NEGATIVE
ORGANISM	NO GROWTH

Comments

URINE SAMPLE RECEIVED ON 19.12.2025.

BLOOD CULTURE (OVERNIGHT)

PRELIMINARY REPORT	BLOOD (RIGHT PERIPHERAL SITE)
ORGANISM	STERILE AFTER OVERNIGHT INCUBATION AT 37°C.
BLOOD CULTURE 48HRS REPORT	RESULT PENDING
BLOOD CULTURE FINAL REPORT	RESULT PENDING
SUSCEPTIBILITY: GRAM POSITIVE ORGANISMS	RESULT PENDING
FIRST LINE ANTIBIOTICS	RESULT PENDING
SECOND LINE ANTIBIOTICS	RESULT PENDING
THIRD LINE ANTIBIOTICS	RESULT PENDING
SUSCEPTIBILITY: GRAM NEGATIVE ORGANISMS	RESULT PENDING
FIRST LINE ANTIBIOTICS	RESULT PENDING
SECOND LINE ANTIBIOTICS	RESULT PENDING
THIRD LINE ANTIBIOTICS	RESULT PENDING
ANTIBIOTIC SUSCEPTIBILITY	RESULT PENDING

CULTURE; NASOPHARYNGEAL

SPECIMEN SOURCE	RIGHT NASAL SWAB
ORGANISM	NORMAL NASAL FLORA GROWN.

CULTURE; NASOPHARYNGEAL

SPECIMEN SOURCE	LEFT NASAL SWAB

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ULR No.1300000245067 7-0013

MC-5835

PATIENT NAME : MR.RAJEEV KAKKAR | **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ, NEW DELHI 110070 11-42776222	ACCESSION NO : **0013YL005861** PATIENT ID : FH.10693597 CLIENT PATIENT ID: UID:10693597 ABHA NO :	AGE/SEX :63 Years Male DRAWN :18/12/2025 20:24:00 RECEIVED : 18/12/2025 20:54:47 REPORTED :21/12/2025 11:25:50

CLINICAL INFORMATION :

UID:10693597 REQNO-24041022
IPD-ICU(2)
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Preliminary**	Results	Biological Reference Interval	Units
ORGANISM	NORMAL NASAL FLORA GROWN.		
AEROBIC CULTURE			
SPECIMEN SOURCE	RIGHT GROIN SWAB		
ORGANISM	NORMAL SKIN FLORA GROWN.		
AEROBIC CULTURE			
SPECIMEN SOURCE	LEFT GROIN SWAB		
ORGANISM	NORMAL SKIN FLORA GROWN.		

Interpretation(s)
CULTURE; URINE-Urinary tract infections (UTI) is defined as bacteriuria, that is multiplication of the organisms in urinary tract and the presence of more than 100,000 organisms per ml, in mid-stream sample of urine. Pyuria denotes the presence of pus cells in urine, which most often accompanies UTI.
Asymptomatic UTI or "covert bacteriuria" can be detected only by urine culture. However in symptomatic patients, a smaller number of bacteria (100 to 10000/mL) may signify infection. In urine specimen obtained by suprapubic aspiration or "in- and-out" catheterization and in samples from a patient with an indwelling catheter, colony counts of 100 to 10000/mL generally indicate infection.
Pyuria in the absence of bacteriuria (sterile pyuria) may indicate infection with unusual bacterial agents such as Chlamydia trachomatis, Ureaplasma urealyticum and Mycobacterium tuberculosis or with fungi, in noninfectious urologic condition such as calculi, anatomic abnormality, nephrocalcinosis vesicoureteral reflux, interstitial nephritis, or polycystic disease.
False negative culture may be due to use of antimicrobial agents, total obstruction below the infection, infection with a fastidious organism, renal tuberculosis and diuresis.
Pus cell, Epithelial cells and RBC counts are based on direct uncentrifuged mount and not from the centrifuge sediment. A urine sample collection in Boric acid stabilizer has an advantage of preventing unwanted growth due to its bacteriostatic action against Bacteria, Fungi in urine.
All culture isolates are maintained for a period of 7 days to facilitate additional test, if required.
CULTURE; NASOPHARYNGEAL-CULTURE; NASOPHARYNGEAL
A nasopharyngeal culture is used to identify pathogenic (disease-causing) bacteria present in the nasal cavity that may cause upper respiratory tract symptoms. Pathogenic organisms that might be identified include Group A beta-hemolytic streptococci, Corynebacterium diphtheriae, Staphylococcus aureus (including Methicillin resistant strains). Additionally bacteria that are considered pathogenic when present in large numbers, include Haemophilus influenzae, Streptococcus pneumoniae, Candida albicans etc.
Nasopharyngeal culture is also used to screen for nasal carriage of Staphylococ aureus.
Results should be correlated clinically.

All culture isolates are maintained for a period of 7 days to facilitate additional test, if required.
AEROBIC CULTURE-
Clinical information inputs play an important role in assisting the microbiologist, to correctly differentiate potential pathogens from normally occurring microorganisms and other contaminants. The final interpretation can be made by the clinician in consultation with the microbiologist on the basis of the clinical correlation. Speciation of all isolates recovered after culture of the specimen is carried out using conventional/automated method of identification.
All culture isolates are maintained for a period of 7 days to facilitate additional test, if required.

****End Of Report****
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ULR No.1300000245067 7-0013

MC-5836

PATIENT NAME : MR.RAJEEV KAKKAR | **REF. DOCTOR :**

FLT LT RAJAN DHAL CHARITABLE TRUST IPD
SECTOR-B, POCKET-1, ARUNA ASAF ALI
MARGVASANT KUNJ,
NEW DELHI 110070
11-42776222

ACCESSION NO : **0013YL006675**
PATIENT ID : FH.10693597
CLIENT PATIENT ID: UID:10693597
ABHA NO :

AGE/SEX : 63 Years Male
DRAWN : 21/12/2025 14:04:00
RECEIVED : 21/12/2025 14:11:31
REPORTED : 21/12/2025 15:54:14

CLINICAL INFORMATION :

UID:10693597 REQNO-24070484
IPD-HDU
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Preliminary**	Results	Biological Reference Interval	Units

MICRO BIOLOGY

GRAM STAIN

SPECIMEN SOURCE	SPUTUM
METHOD : MICROSCOPIC EXAMINATION	
GRAM STAIN	PUS CELLS : 10-25/ LPF SQUAMOUS EPITHELIAL CELLS : <10/ LPF QUALITY SCORE : 1 SMEAR SHOWS GRAM POSITIVE COCCI IN PAIRS, SHORT CHAINS AND FEW GRAM NEGATIVE BACILLI.
METHOD : MICROSCOPIC EXAMINATION	
SPUTUM CULTURE	RESULT PENDING

End Of Report

Please visit www.agilusdiagnostics.com for related Test Information for this accession

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ULR No.23000002451621-0013